

# METROPOLITAN GENERAL HOSPITAL

2100 Healing Way, Boston, MA 02115 | (617) 555-0800 | www.metrogen.org

## DISCHARGE SUMMARY

|                                                               |                                                       |
|---------------------------------------------------------------|-------------------------------------------------------|
| <b>Patient Name:</b> Isabella Santos                          | <b>DOB:</b> January 21, 1996 (Age 54)                 |
| <b>MRN:</b> MRN-8339571                                       | <b>Gender:</b> Male                                   |
| <b>Admission Date:</b> April 29, 2025                         | <b>Discharge Date:</b> June 05, 2025 (3 days)         |
| <b>Attending Physician:</b> Dr. Priya Desai, MD — Pulmonology | <b>Insurance:</b> BlueCross BlueShield #BCB-779-00234 |

## DIAGNOSES

| Primary Diagnosis                            | ICD-10 Code | Status                 |
|----------------------------------------------|-------------|------------------------|
| Iron Deficiency Anemia                       | D50.9       | Resolved               |
| Hyperlipidemia                               | E78.5       | Chronic / Pre-existing |
| Acute Kidney Injury, Stage 1                 | N17.9       | Resolved               |
| Chronic Obstructive Pulmonary Disease (COPD) | J44.9       | Chronic / Pre-existing |

## HOSPITAL COURSE

Isabella, a 54-year-old male with a known history of Type 2 Diabetes and Hypertension, presented to the Emergency Department on April 29, 2025 via ambulance with a 2-hour history of severe substernal chest pain radiating to the left arm, diaphoresis, and nausea. Initial ECG demonstrated ST-segment elevation in leads V1–V4 consistent with anterior STEMI. Troponin-I on arrival was critically elevated at 24.9 ng/mL. The patient was taken emergently to the cardiac catheterization laboratory where coronary angiography revealed 91% occlusion of the left anterior descending (LAD) artery. Successful percutaneous coronary intervention (PCI) was performed with deployment of a drug-eluting stent (Xience Skypoint 3.0 x 28mm). Post-procedural TIMI flow was grade 3. The patient was transferred to the Cardiac Care Unit (CCU) for monitoring. Serial echocardiography on day 2 showed an ejection fraction of 42%, mild anterior wall hypokinesis. Mild acute kidney injury (Creatinine 1.4 mg/dL) was noted, attributed to contrast nephropathy; this resolved with IV hydration by day 4. Blood glucose remained elevated throughout admission and endocrinology was consulted to optimize insulin regimen. The patient remained hemodynamically stable throughout and was discharged on day 3 in stable condition with close outpatient follow-up arranged.

## VITALS AT DISCHARGE

| BP          | HR     | SpO2     | Temp   | RR     | Weight |
|-------------|--------|----------|--------|--------|--------|
| 116/85 mmHg | 81 bpm | 96% (RA) | 98.0°F | 16/min | 104 kg |

## DISCHARGE MEDICATIONS

| Medication                | Dose     | Frequency              | Duration                   |
|---------------------------|----------|------------------------|----------------------------|
| Aspirin                   | 81 mg    | Once daily             | Indefinite                 |
| Clopidogrel (Plavix)      | 75 mg    | Once daily             | 12 months                  |
| Atorvastatin              | 80 mg    | Once at bedtime        | Indefinite                 |
| Metoprolol Succinate      | 50 mg    | Once daily             | Indefinite                 |
| Lisinopril                | 10 mg    | Once daily             | Indefinite                 |
| Insulin Glargine (Lantus) | 25 units | Once daily at bedtime  | Ongoing — follow with Endo |
| Metformin                 | 500 mg   | Twice daily with meals | Ongoing                    |

## FOLLOW-UP PLAN

**Cardiology:** Dr. Forsythe — 1 week post-discharge. Repeat Echo at 6 weeks.  
**Endocrinology:** Dr. Patel — 2 weeks post-discharge for diabetes management.

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**Primary Care:** Dr. Kim — 4 weeks. Renal function panel to confirm AKI resolution.  
**Cardiac Rehab:** Enrollment arranged — starts July 13, 2025 at MetroGen Rehab Center.  
**Labs:** BMP, HbA1c, Lipid Panel — 2 weeks post-discharge.

**Attending Physician Signature:** \_\_\_\_\_  
Dr. Priya Desai, MD — Pulmonology | License #: MA-MD-42273 | Date: June 05, 2025  
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